

M7T3: Monitoring Progress and Adjustments — 11 min

Video File M7T3-Monitoring.mp4 (progress dashboard example) · **Resources** Progress Tracker.xlsx (weekly dashboard: energy, sleep, mood, cravings, cycles, habits; adjustment log; referral checklist) · **Xp** 100 total · Monitor 1 Week 50 · Adjust Plan 30 · Share 20

Target runtime 11 min · 1,204 spoken words · ≈ 8.0 min at 150 wpm · ≈ 8.9 min at a warm 135 wpm

0:00–0:30 **HOOK**

[VISUAL: Progress dashboard example on screen: a simple weekly grid with a few lines trending gently up.]

You've tracked cravings, mood, energy, sleep, steps, calm scores, and bond scores across six modules. That's a lot of numbers, and I want to tell you something about them: most of them don't matter anymore. This training is about the five that do, how to read them without obsessing, and, just as important, the point at which numbers stop being enough and it's time to sit down with a specialist.

0:30–4:30 **TOOLS: WHAT TO TRACK**

[VISUAL: Slide: "Self-monitoring is the single most effective behavior-change technique (Health Psychology)."]

The science of monitoring is unambiguous. Susan Michie and colleagues reviewed more than a hundred interventions in the journal *Health Psychology* and found that self-monitoring, combined with goal-setting and feedback, was the most

effective technique for changing eating and activity. Lora Burke's review in the Journal of the American Dietetic Association, which we met in Module One, found the same for eating. Tracking works. But tracking everything is its own kind of stress, and this is a stress-reduction course. So we track five things, weekly, in the Progress Tracker.

[VISUAL: Slide: "The five: energy · sleep · mood · cravings · cycles. Plus the habit count."]

Energy, at three in the afternoon, one to ten, averaged over the week. Sleep, hours and quality. Mood, at nine at night, one to ten. Cravings, how many days this week a craving won, honestly. And cycles, for women: cycle length, and any changes in regularity, flow, or pain; for men, nothing to track here except the sauna rule. Plus the habit count from the Habit Log: how many of your fifteen you kept this week. That's it. Six rows, seven days, on a spreadsheet you fill in on Sunday in three minutes.

[VISUAL: Slide: "What we're not tracking: weight (unless your doctor asks) · calories · steps beyond 'walked' · anyone else's numbers."]

What we're not tracking: weight, unless your doctor has specifically asked you to. Weight is a poor short-term signal, it swings with water and cycle, and it's the number most likely

to trigger the old shame. If a healthy weight is part of your plan, and for some conditions it matters, the habits are how you get there, and your doctor can track the number. Not calories, ever. Not steps beyond "walked or didn't." And not anyone else's numbers in the forum, because comparison is the thief of this entire module.

The tracker has a dashboard tab that turns the six rows into six lines. You're looking for direction over a month, not for a good week.

A few words on reading cycles specifically, because it's the line women ask about most. A cycle between roughly twenty-one and thirty-five days, arriving within a few days of the same length each time, is the pattern the reproductive medicine bodies describe as regular. If yours has been longer, shorter, or unpredictable and starts to settle over two or three months of the plan, that's a meaningful change, and worth telling your doctor about at your next visit. If it doesn't settle, or if you have no cycle at all, that's one of the "don't wait" signs in the referral list, not something the plan is expected to fix on its own. The tracker never replaces that conversation; it makes it more useful, because you'll arrive with three months

of data instead of a guess. Energy drifting up. Cravings drifting down. Sleep steadying. Cycles becoming more regular, which is one of the most encouraging signals a woman can see, because regularity is the outward sign that the hormonal rhythm underneath is settling.

4:30–8:30 ADJUSTMENTS

[VISUAL: Slide: "Adjust when: a line goes the wrong way for two weeks · a habit hasn't stuck in a month · life changes."]

Adjust, don't abandon. Three triggers for adjustment. First, a line going the wrong way for two weeks: energy dropping, cravings rising, sleep fragmenting. Go to the pillar that line belongs to and look at the habit count for it; the answer is usually there, a protein breakfast that slipped, a bedtime that drifted. Second, a habit that hasn't stuck after a month of the log: don't try harder, make it smaller, or change the anchor, or drop it for a different habit in the same pillar. Third, life changing: a new job, a move, illness, treatment, a pregnancy. The plan is a living document; the adjustment log in the tracker records each change and why, so that in three months you can see how you steered.

[VISUAL: Slide: "Reread your pre-course survey."]

And here's the adjustment I most want you to make. Go back to your pre-course survey: your big why, the support you had, your heartfelt goals, and what you said you'd do, be, and change. Read it now, six weeks on. Some goals you've met. Some have changed shape; that's growth, not failure. Some you wrote before you knew what you know now. Rewrite them in the tracker, in the light of the data, and notice, please notice, how far the person who wrote the original has come. Progress is nonlinear, and it is real, and the survey is the proof.

[VISUAL: Slide: "For couples: two dashboards, one conversation."]

For couples: two dashboards, compared at the weekly check-in, and I'll repeat Module Six's rule: reflect before you react. If one partner's lines are moving and the other's aren't, that's a "how can we support each other" conversation, not a "why aren't you trying" one.

8:30–10:00 **WHEN TO CONSULT A PROFESSIONAL**

[VISUAL: Slide: "ASRM / ACOG: seek evaluation after 12 months of trying under 35 · 6 months at 35+ · sooner at 40+ or with known conditions."]

Now the part I most want you to hear clearly, because a wellness course should never delay medical care. The

American Society for Reproductive Medicine and the American College of Obstetricians and Gynecologists give the same guidance: if you're under thirty-five and have been trying for twelve months without conceiving, see a doctor or a fertility specialist. If you're thirty-five or older, six months. If you're forty or older, or if there's any known issue, don't wait at all. Known issues include irregular or absent periods, a diagnosis of PCOS, endometriosis, or thyroid disease, two or more pregnancy losses, a history of pelvic infection, or, for men, any known testicular or sperm problem, or a history of chemotherapy. If any of those apply, the four-month plan runs alongside the specialist, not instead.

And beyond fertility: if the mood line stays low for more than two weeks, Module Four's guidance applies; if sleep stays broken for a month despite the routine, ask your doctor; if cycles change dramatically, tell your doctor. None of this replaces the plan. The plan is what makes every one of those conversations start from a healthier baseline, and specialists notice.

Celebrate the generational impact, even here. Choosing to ask for help on time is one of the most protective things a future parent can do.

I'll add one thing for the partner who tends to say "let's give it a bit longer." The referral timelines exist because evaluation is not the same as treatment; a first appointment is usually blood tests, a semen analysis, and an ultrasound, and the most common result is reassurance and a clearer plan. Waiting past the timeline doesn't protect you from anything. It only delays information. Going on time, with your dashboard and your plan in hand, is what a well-prepared couple does, and specialists tell me they can see the difference in the first five minutes.

10:00–11:00 HEART-LED CLOSE AND ACTION HANDOFF

[VISUAL: End card: "Progress Tracker XLS → Monitor one week (50 XP) → Adjust your plan (30 XP) → Share a screenshot (20 XP)."]

This is for your future family, and monitoring isn't surveillance; it's attention, the same attention you'll one day pay to a child's sleep and moods. Open the Progress Tracker, monitor one full week for fifty XP, make one adjustment to your plan and log why for thirty, and share a screenshot of

your dashboard in the forum for twenty. Next, Training Four: how to celebrate, and why the celebration is what makes the habit stay.

ACTION CALLOUT

RESOURCE Progress Tracker.xlsx (weekly six-row log, dashboard tab, adjustment log, pre-course survey revisit page, specialist referral checklist). **ACTION 1:** Monitor one full week (50 XP). **ACTION 2:** Make one adjustment and log why (30 XP). **ACTION 3:** Share a dashboard screenshot in the forum (20 XP). **NEXT STEP:** Training 4 · Celebrating Milestones.